جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ايمان سعيد السيد ابوالعنين
المحافظة : المنوفيه **الحي / المدينة :** الباجور **العنوان :** ابو سنيطه الباجور
رقم البطاقة : 29511051700442 **رقم التليفون :** 01061069353 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 05-11-1995 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي مثبط CDK4/6 و فلفسترانت [14469] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ايمان سعيد السيد ابوالعنين
National ID: 29511051700442

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي CDK4/6 inhibitor and fulvestrant

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





DMS

ابيراش فازلودكس

زوعدكس

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient's Name:	**Iman Sa**
Date:	**2026-06-٠٤**
Referred by:	Prof.Dr. no doctor
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: **Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.**

FINDINGS:

Clinically reported right breast carcinoma compared to last study dated on 13/1/2026 .

(I) Chest:

- **Still noted Right breast heterogenous parenchyma with nipple retraction**
- **Middle lobe medial segment area of ground glass attenuation likely infectious .**
- **No sizable hilar or mediastinal lymph nodes .**
- **No sizable pulmonary nodules .**
- **Clear both lung fields .no sizable pulmonary masses or consolidation patches**
- **No pleural or pericardial collections seen.**
- **No gross cardiac abnormality.**
- **Azygos lobe and azygos fissure**

(II) Abdomen and pelvis:

- **The liver is enlarged in size showing homogenous CT density , with the previously described bilobar focal lesions not barely appreciated at current study .**
- **Patent and homogenously enhancing portal vein.**
- **No intra or extra hepatic biliary radicles dilatation.**
- **Normal CT appearance of the pancreas, both kidneys, aorta and IVC.**
- **Normal CT appearance of the spleen .**
- **Regular filling of the urinary bladder showing no stones, masses .**
- **Unremarkable other pelvic organs.**
- **No retro-crural, para-aortic or pelvic lymphadenopathy.**
- **No ascites.**

Skeletal system :

- **Spondylodegenerative changes .**
- **still noted axial and appendicular skeleton sclerotic lesions .**
- **No vertebral structural collapse .**

OPINION:.

- **Middle lobe medial segment area of ground glass attenuation likely infectious Mottled appearance of examined bones with L2 small lytic lesion for correlation with bone scan**
- **previously described bilobar focal lesions not barely appreciated at current studyfor dynamic MRI .**
- **rather stationary rest of the study**

Dr.Reham Khedr, MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Pat. Name: Eman Saeed Al Sayed	Pat. ID: 3258287
Procedure Name: Bone Scan	Date: 07/05/2026

Tc-99m MDP Follow-Up Bone Scan

Compared to the last bone scan dated 18.2.2026 , the current study revealed mild regression in tracer uptake intensity of the previously detected active bone lesions involving multiple cervico-dorso-lumbar vertebrae , left posterior parietal skull bone, multiple bilateral scattered ribs, sternum , left humeral head , left scapula , multiple bilateral pelvic bones and proximal femora.

NO newly developed active or cold osseous lesions detected.

OPINION:

Mild regression of some of the old bone metastases

Best Regards,
Prof. Dr. Maha Abd Elkareem
Professor of Nuclear Medicine
Kasr Al Ainy, Cairo University

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

tient ID: | lame : ايمن سعيد السيد ابو الغين | Sex: Female | J- Medical number: 83214924 | Old Med : | 2006-2024

number: 29511051700442 | Patient Age : 29 Y

MDT

fy history		Phone	01002750595	BMI		REASON OF MDT	
tal status		NRS Score		GROUP		Date of MDT	13/08/2024
orbidity		Menopausal status		Initial STAGE			

Clinical data

ast Imaging :

Mets Workup :

	ER	PR	Her 2 Neu	SISH	Ki67
	8/8	0/8	0		7
			-ve		

	N	M
		bone

omammography

IENT HISTORY

tially MBC to bone
rted tam+zoladex 5/2023 ti 4/2024
al progression --> started aramdix+zoladex
6/2024: PD.
T & bone scan 7/2024: disease progression

MDT OUTCOME & PLAN

TCOME & PLAN

faslodex , zoladex , ibrance
meta

SIGNATURE

DICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
RGERY	SURGERY	PATHOLOGY	PATHOLOGY
chiatric	Psychiatry		

MDT Form [[DOCTYPE:MDT_FORM]]

Pat. Name: Eman Saeed Al Sayed	Pat. ID: 3258287
Procedure Name: Bone Scan	Date: 18/02/2026

Tc-99m MDP Follow-Up Bone Scan

Compared to the last bone scan dated 3/9/2025 , the current study revealed mild to moderate regression in tracer uptake intensity of the previously detected active bone lesions involving the spine and ribs.
Rather stationary course of the other ole lesions involving the sternum , left humeral head , pelvic bones and proximal femora.
NO newly developed active or cold osteous lesions detected.

OPINION:

Partial regression of some of the old lesions , stationary course of the other old lesions and no newly developed active or cold bone metastases detected.

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Name : EMAN SAEED EL SAYED	Date : 13-01-2026 10:53
Branch : Portable	Medical No. : GRI-29511051700442
Ref.Physician Prof :	Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA with no available previous images for comparison.

I- CHEST:

- Heterogenous right breast parenchyma with minimal skin thickening for sono-mammography.
- Azygos lobe and fissure.
- No pulmonary nodules, masses or cavitary lesions.
- Few bilateral scattered atelectatic bands.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. Few tiny bilobar hypodense focal lesions largest measuring 7.2 x 4.8 mm at segment VIII. No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple scattered axial and appendicular skeleton sclerotic areas.

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Name : EMAN SAEED EL SAYED	Date : 13-01-2026 10:53
Branch : Portable	Medical No. : GRI-29511051700442
Ref.Physician Prof :	Ins.Company :

Opinion:

- Known case of breast CA showing:
  - Few tiny bilobar hypodense hepatic focal lesions for dynamic MR study.
  - Multiple axial and appendicular skeleton sclerotic areas for bone scan.

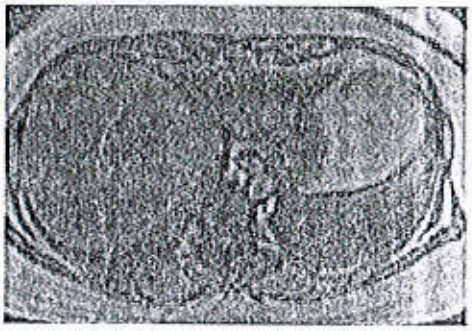
Dr. Sherif Wagdy

Dr. Sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name :-	ايمان محمد السيد ابوالعزايم
Referred by :Prof. Dr :-	
Date :-	11/2/2026

MULTIPLANNER, MULTIPARAMETRIC, DYNAMIC MRI SCAN OF THE ABDOMEN REVEALED:-



- Normal hepatic size & fatty parenchymal texture, with no IHBRD seen. No pathologically significant portahepatis LNs. Segment VII hepatic small lesion measures 10mmx11mmx10mm is seen. The lesion is isointense to rest of the liver on all pulse sequences except the out off phase segmence when it appears relatively hyperintense to rest of the liver. On post contrast dynamic series no detectable areas with abnormal enhancement patter.
- Patent PV, splenic vein and SMV.
- Normal size shape and texture of the spleen.
- Normal gall bladder shows no intraluminal signal void stones. Normal CBD.
- Normal MRI appearance of the pancreas, both suprarenal glands, both kidneys, aorta and IVC.
- No retrocrural or para-aortic lymphadenopathy seen.
- No ascites.
- Normal imaged bowel structures.
- Free lower thoracic cuts.

OPINION:

- **Fatty liver with right lobe ?? small focal fat sparing.**
- **Normal MRI study of the other abdominal organs.**

Dr. Mohamed El Sayed MD

M.R.I
[[DOCTYPE:MRI]]

Patient Name	: الاسكندر / ايمان سعيد ابو العينين	Request No.	: 13003419
Patient No.	: 29511051700442	Request Date	: 16/07/2024
Path No.	: 2422/24	Verification Date	: 05/08/2024
Sex	: Female	Financial Category	: اوراما مصر السلام - تغطية اضافية
Age	: 28 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: imaan mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 15/7/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of the tumor cells (score 8/8).
- PR: No nuclear reaction in the tumor cells (score 0/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 7% nuclear reaction of tumor cells.

Conclusion:
Right breast swelling, Tru-cut biopsy:
invasive lobular carcinoma, Grade II, with focal perineural invasion.
IPT:
- ER: Positive (score 8/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67: Positive in 7% of tumor cells.

- Scoring system: "ER, PR"

Verified by :

شقة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الاسكندراني / ايمان سعيد ابو الدهب	Request No.	13003419
Patient No.	29511051700442	Request Date	16/07/2024
Path No.	2412/24	Verification Date	05/08/2024
Sex	Female	Financial Category	زوار غير السلام - عقد قديم
Age	28 Y	Department	Out Patient
Referreing Doc.		Registered by	marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر الجديدة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

[[DOCTYPE:CT_SCAN]]
CT Scan

Medical No. : 2831
Patient's Name: Iman Said El-Sayed
Date: 2026-09-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

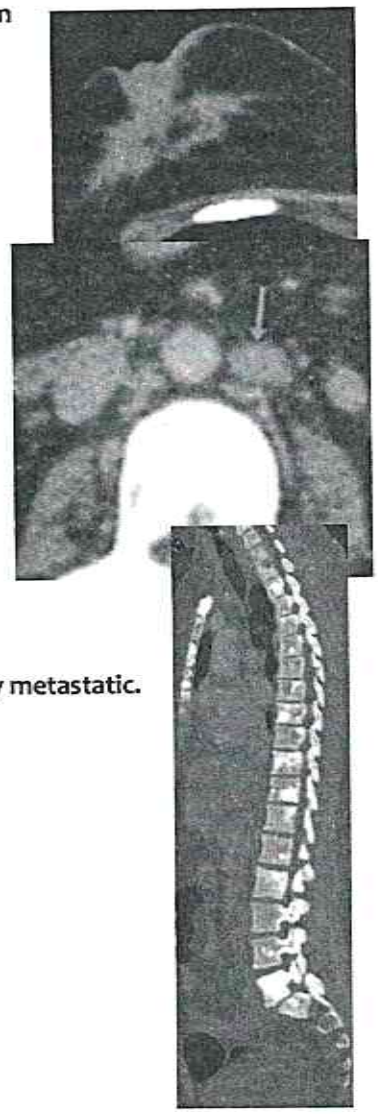
Clinically reported right breast carcinoma compared to last study dated on 1/6/2026.

(I) Chest:

- **Still noted right breast dense irregular parenchyma with nipple retraction**
- **Middle lobe and lingular thick parenchymal bands.**
- No sizable hilar or mediastinal lymph nodes.
- No sizable pulmonary nodules, masses or consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Azygos lobe and azygos fissure (variant).

(II) Abdomen and pelvis:

- The liver is of average size showing homogenous CT density, with no sizable suspicious focal lesions in the current study.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- Normal CT appearance of the spleen.
- **Evident enlarged para-aortic LNs in the current study, the largest seen on the left, oval in shape, measuring 16x10mm.**
- Regular filling of the urinary bladder showing no stones, masses .
- **IUD seen in place.**
- No ascites.



Skeletal system:

- **Still noted axial and appendicular skeleton diffuse sclerotic lesions likely metastatic.**
- **No vertebral structural collapse.**

OPINION:

Follow up study of a known patient with right breast cancer, study reveals:

- **No suspicious pulmonary nodules.**
- **Evident enlarged para-aortic LNs, likely reactionary for follow up.**
- **Diffuse skeletal metastasis.**

Much obliged,
Shohenda. M. Taha, MD

Dr.Rahma Khader

Dr.Rahma Khader

[[DOCTYPE:BONE_SCAN]]
Bone Scan

PATIENT NAME: EMAN SAYED ELSAYYED ABOU EL ENIN **GENDER:** Female **DATE :** 15-Sep-2026 10:34 AM	**PATIENT ID:** 10123224100 **AGE:** 30 y **ORDER:** 3931404

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP:

Report:

Compared to previous bone scan report dated 7/5/2026, the current study revealed:-
More or less stationary course of the wide spread metastatic bone deposits at posterior parietal skull bones, cervical, dorsal and lumbar vertebrae, multiple bilateral ribs, sternum, right Scapula, multiple pelvic bones, sacrum, proximal both femora and both shoulder regions.
Currently seen tiny warm uptake at the right parietal and occipital skull bones for follow up.

Both kidneys are visualized.

Conclusion:

Follow up bone scan shows rather stationary course of the metastatic osseous deposits, compared to last report dated 7/5/2026.

Reported By,

Dr. Huda Fathy Elsayed Abdulhalim **15-Sep-2026 03:43 PM**

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16602
مركز بهية الهرم
4 شارع علوية أول شارع الهرم بجوار داري , الجيزة
مستشفى بهية الشيخ زايد
٤ - الحي الأول - الشيخ زايد أمام كلية الهندسة